

Pityriasis Alba (PA)

Pityriasis alba (PA) typically presents as a pink patch with an elevated border, which fades after several weeks into a paler spot covered with fine, powdery white scale. Lesions eventually progress to non-scaly hypopigmented macules that may persist for months or years. This hypomelanosis results from a decreased number of melanocytes and melanosomes. The three stages—erythematous, scaling, and hypopigmented—may occur simultaneously in the same patient. Most cases of PA are diagnosed clinically. Histopathology is non-specific.

Extensive Pityriasis Alba

Extensive PA, more commonly observed in adults, exhibits the characteristic features of PA but is distributed more broadly and symmetrically, often involving the lower trunk. Unlike classic PA, it lacks a preceding inflammatory phase and does not show spongiosis histologically. These differences help distinguish extensive PA from the classic form. There may be an overlap between extensive PA and progressive macular hypomelanosis—a condition primarily reported in young adult females, characterized by recurrent, hypopigmented, non-scaling patches on the back, often appearing after sun exposure.

Pigmenting Pityriasis Alba

Pigmenting PA is a rare variant in which classic PA coexists with a superficial dermatophyte infection, almost always affecting the face. It presents clinically with a central bluish hyperpigmented area surrounded by a hypopigmented, scaly rim. The hyperpigmentation is due to dermal melanin deposition. Approximately one-third of patients with pigmenting PA also have concurrent classic PA.

Differential Diagnosis

Differential diagnoses for PA include all localized forms of hypopigmentation, particularly: - Post-inflammatory hypopigmentation (e.g., following psoriasis or eczema) - Fungal infections (e.g., tinea versicolor) - Nevus depigmentosus - Nevus anemicus - Tuberous sclerosis - Mycosis fungoides (MF) - Vitiligo

Management

PA is self-limiting, and treatment is often only partially effective. Supportive therapies include: - Topical corticosteroids (for active, inflamed lesions) - Emollients - Topical tretinoin (may be effective but potentially irritating)

The less common variants show better treatment responses: - **Extensive PA:** Responds to ultraviolet (UV) therapy - **Pigmenting PA:** Responds to oral antifungals

Supportive Measures

- Reduce sun exposure
- Use broad-spectrum sunscreens
- Decrease frequency and temperature of baths to prevent skin dryness

Sarcoidosis and Hypopigmentation

Hypopigmentation is a rare cutaneous manifestation of sarcoidosis. It presents as hypopigmented macules scattered on the trunk and extremities; papular or nodular lesions may also occur. Diagnosis is supported by the presence of noncaseating dermal granulomas, best identified in biopsies of indurated lesions. Histologically, there is reduced epidermal melanin content with preservation of melanocytes.

The exact pathogenesis of pigment loss in sarcoidosis remains unclear.